

ADMISSION NOTE - Patient synth_03 - Enteric fever (Typhoid) with fever spikes

UNITY MULTISPECIALITY HOSPITAL, DELHI

IP NO: 26042103

PATIENT NAME: Arjun Sharma, 24/M

ADMISSION DATE: 21/04/26

CONSULTANT: Dr. R. Verma

DIAGNOSIS:

? Enteric Fever

Dehydration

? Dengue Fever

CHIEF COMPLAINTS:

High-grade fever for 5 days.

Headache and body ache for 5 days.

Loss of appetite.

HISTORY OF PRESENT ILLNESS:

Patient is a 24-year-old male who was apparently well 5 days ago, after which he developed high-grade fever, documented up to 103°F. Fever is intermittent, step-ladder pattern, not associated with chills or rigors. This is associated with a severe, throbbing frontal headache, generalized body ache, and profound anorexia. He took over-the-counter TAB DOLO-650 which provided temporary relief of fever and pain. No history of cough, coryza, loose stools, vomiting, abdominal pain, or skin rash. He gives a history of consuming food and beverages from a street vendor approximately 10 days prior to the onset of symptoms. He denies any recent travel.

PAST HISTORY:

No known co-morbidities. Not a K/C/O Diabetes Mellitus, Hypertension, CAD, Asthma or Thyroid disorder. No history of previous surgeries or hospitalizations. No known drug allergies.

PERSONAL HISTORY:

Occupation: Software Engineer. Diet: Mixed. Habits: Non-smoker, occasional alcohol consumption. Sleep: Disturbed due to fever.

GENERAL EXAMINATION:

Patient is conscious, oriented to time, place, and person. Appears tired and toxic.

VITALS: Temp: 102.8°F, PR: 110/min (tachycardia), BP: 110/70 mmHg, RR: 18/min, SpO2: 99% on room air.

Pallor is present. No icterus, cyanosis, clubbing, lymphadenopathy or pedal edema.

SYSTEMIC EXAMINATION:

CARDIOVASCULAR SYSTEM: S1, S2 heard. No murmurs.

RESPIRATORY SYSTEM: Bilateral air entry equal. No added sounds.

PER ABDOMEN: Soft, non-tender. Bowel sounds heard. No hepatosplenomegaly.

CENTRAL NERVOUS SYSTEM: GCS 15/15. No focal neurological deficits. Neck soft.

INVESTIGATIONS:

Blood samples sent for: Complete Blood Count, Liver Function Test, Kidney Function Test, Widal Test, Typhidot IgM, Dengue NS1/IgM/IgG, Blood Culture & Sensitivity. Urine R/M. Reports are awaited.

PLAN:

- Admit to ward.
- Start IV fluids (Normal Saline).
- INJ MONOCEF 1g IV BD.
- TAB PAN-40 40mg OD.
- TAB DOLO-650 SOS for fever >100°F.
- INJ EMESET 4mg IV SOS for nausea/vomiting.
- Monitor vital signs and urine output.

PROGRESS NOTE ? Day 1 - Patient synth_03 - Enteric fever (Typhoid) with fever spikes

UNITY MULTISPECIALITY HOSPITAL, DELHI
PROGRESS NOTE

PATIENT NAME: Arjun Sharma, 24/M
DATE: 22/04/26

Subjective:

Patient reports feeling slightly better. Fever spikes continue, but were less intense overnight, highest recorded was 101°F. Headache has reduced significantly. Tolerating sips of water and oral fluids. One episode of nausea, no vomiting.

Objective:

General condition appears stable. Patient is conscious and oriented.

VITALS: PR 96/min, BP 116/70 mmHg, RR 18/min, SpO2 98% RA, Temp 100.2°F.

Urine output is adequate.

Investigations:

- CBC: TLC 3,800/cumm (Leucopenia), Platelets 1.8 Lakhs/cumm.
- LFT: SGOT 78 U/L, SGPT 85 U/L (mildly elevated).
- Serology: Widal Titre - TO > 1:160, TH > 1:160. Typhidot IgM is POSITIVE.
- Dengue NS1 Antigen: Negative.
- Blood Culture & Sensitivity: Report awaited.

Assessment:

Confirmed case of Enteric Fever (Typhoid). Responding to current line of treatment. Leucopenia is consistent with diagnosis.

Plan:

- Continue current management.
- INJ MONOCEF 1g IV 1-0-1.
- TAB PAN-40 40mg 1-0-0.
- TAB DOLO-650 SOS for fever.
- Encourage good oral hydration and soft diet as tolerated.
- Continue to monitor vitals closely.

PROGRESS NOTE ? Day 2 - Patient synth_03 - Enteric fever (Typhoid) with fever spikes

UNITY MULTISPECIALITY HOSPITAL, DELHI
PROGRESS NOTE

PATIENT NAME: Arjun Sharma, 24/M
DATE: 23/04/26

Subjective:

Patient is feeling significantly better today. He has been afebrile for the last 20 hours. No headache or body ache. Appetite has improved, and he has taken breakfast. No new complaints.

Objective:

Patient looks comfortable and well-hydrated. General condition has improved markedly.

VITALS: PR 84/min, BP 120/80 mmHg, RR 16/min, SpO2 99% RA, Temp 98.6°F afebrile.

Abdomen is soft and non-tender. Accepting oral diet well.

Investigations:

- Repeat CBC: TLC 4,500/cumm, Platelets 2.1 Lakhs/cumm. Shows improvement.

Assessment:

Enteric Fever, resolving. Patient is hemodynamically stable and clinically fit for discharge. He can be managed on oral antibiotics at home.

Plan:

- Plan for discharge tomorrow (24/04/26).
- Stop IV antibiotics and IV line.
- Switch to oral antibiotics: Start TAB AZITHRAL 500mg 1-0-0 for 5 days.
- Continue TAB PAN-40 and TAB DOLO-650 SOS.
- Explain discharge advice regarding diet, rest, and medication adherence.
- Prepare discharge summary.

LABORATORY REPORT - Patient synth_03 - Enteric fever (Typhoid) with fever spikes

UNITY MULTISPECIALITY HOSPITAL, DELHI
LABORATORY REPORT

PATIENT NAME: Arjun Sharma, 24/M

IP NO: 26042103

COLLECTION DATE: 21/04/26

--- COMPLETE BLOOD COUNT (CBC) ---

PARAMETER VALUE UNITS REFERENCE RANGE

Total Leucocyte Count 3,800 /cumm 4,000 - 11,000

Platelet Count 1.8 Lakhs/cumm 1.5 - 4.5

Hemoglobin 13.5 g/dL 13.0 - 17.0

--- LIVER FUNCTION TEST (LFT) ---

PARAMETER VALUE UNITS REFERENCE RANGE

SGOT (AST) 78 U/L < 40

SGPT (ALT) 85 U/L < 42

Total Bilirubin 0.9 mg/dL 0.2 - 1.2

--- SEROLOGY ---

PARAMETER VALUE RESULT

Widal Test (Slide) TO > 1:160, TH > 1:160 Positive

Typhidot IgM Positive Positive

Dengue NS1 Antigen Negative Negative

--- MICROBIOLOGY ---

PARAMETER STATUS

Blood Culture & Sensitivity Awaited

MEDICATION RECORD - Patient synth_03 - Enteric fever (Typhoid) with fever spikes

UNITY MULTISPECIALITY HOSPITAL, DELHI

MEDICATION RECORD

PATIENT NAME: Arjun Sharma, 24/M

IP NO: 26042103

--- ADMISSION MEDICATIONS (from 21/04/26) ---

1. INJ MONOCEF 1g IV 1-0-1
2. TAB PAN-40 40mg 1-0-0
3. TAB DOLO-650 1 tab SOS (for fever >100°F)
4. INJ EMESET 4mg IV SOS (for vomiting)
5. IV FLUIDS - NS @ 100ml/hr

--- DISCHARGE MEDICATIONS (from 24/04/26) ---

1. TAB AZITHRAL 500mg - 1-0-0 (for 5 days)
2. TAB PAN-40 40mg - 1-0-0 (for 1 week)
3. TAB DOLO-650 650mg - SOS (for fever/pain)
4. TAB BECOSULES 1 cap - 0-1-0 (for 2 weeks)

DISCHARGE ADVICE - Patient synth_03 - Enteric fever (Typhoid) with fever spikes

UNITY MULTISPECIALITY HOSPITAL, DELHI
DISCHARGE SUMMARY

IP NO: 26042103
PATIENT NAME: Arjun Sharma, 24/M
DATE OF ADMISSION: 21/04/26
DATE OF DISCHARGE: 24/04/26
CONSULTANT: Dr. R. Verma

- DIAGNOSIS:
- 1. Enteric Fever (Typhoid Fever)
 - 2. Dehydration (Resolved)
 - 3. Acute Febrile Myalgia

COURSE IN THE HOSPITAL:

Patient was admitted with high-grade fever, headache and body ache. Investigations confirmed diagnosis of Enteric Fever. He was managed with IV antibiotics (INJ MONOCEF), antipyretics and IV fluids. He responded well to treatment, became afebrile and symptomatically better. Now being discharged in a stable condition on oral antibiotics.

CONDITION AT DISCHARGE:

Patient is conscious, alert, and well-oriented. Afebrile, hemodynamically stable. Accepting soft diet and oral fluids well. Vitals: PR 80/min, BP 120/80 mmHg, RR 16/min, Temp 98.4°F.

- ADVICE ON DISCHARGE:
- Take adequate rest for one week.
 - Drink plenty of fluids (water, juice, soup).
 - Eat a soft, easily digestible diet. Avoid oily, spicy, and outside food.
 - Continue the following medications:

No.	Medication	Name	Dosage	Frequency	Duration

1.	TAB	AZITHRAL	500 mg	1-0-0	5 Days
2.	TAB	PAN-40	40 mg	1-0-0	1 Week
3.	TAB	DOLO-650	650 mg	SOS	As needed
4.	TAB	BECOSULES	1 Capsule	0-1-0	2 Weeks

- FOLLOW-UP INSTRUCTIONS:
- Review in the Medicine OPD with Dr. R. Verma after 5 days.
 - Report to the hospital immediately in case of high fever, severe abdominal pain, vomiting, or any other new complaints.

PENDING RESULTS:

- Blood Culture & Sensitivity Report.